

PRE-SEND CHECKLIST — the psychiatric instruction

INTERNAL. This never goes to the practice, to the employer or to the Regulator, and it is not in the pack.

Everything else built for this is written to be produced; this is not. Rewritten and renumbered 16 August 2026; items 0 and 6 rewritten 17 August 2026.

A · BEFORE THE INSTRUCTION IS SENT

0. TYPE THE COVERING EMAIL. DO NOT ATTACH IT. *COVERING_EMAIL_to_practice.pdf* is a template to type from, and it is **not in the pack**. It opens with a production note — “assume this email is produced to the Regulator under the Notice of Non-Party Disclosure; nothing in it may be strategic” — which is a note to you. **If the practice files the PDF, Ms Matheson reads the note with it.** The body of the email goes as written, subject line included. Then check the attachment list against the pack before sending: **twelve files, and no covering email among them** — letter · schedule · Attachments 1, 2, 3 · the four Attachment 4 files · Attachment 5 · Attachment 6 (the sealed Form 29) · Attachment 7 (the 31 July request).

1. Insert the date. It is the only bracket left in the letter.

2. Funding is closed — Metro South Health contacted the practice directly and confirmed it will meet the cost. Not a promise to Cory but a commitment to a third party, of which the practice holds the record.

Two follow-ups, and neither involves contacting MSH: (a) confirm the **figure** with the practice when booking — “will meet the cost” is not “up to \$X”, and re-opening it with MSH invites a cap; (b) ask the practice one neutral question: **what has the Health Service provided to you, or asked of you?** If MSH framed a scope or sent its own questions, the doctor may think he is answering their brief. **The letter of instruction must be the only instruction** — and question 1.5 catches it either way.

3. The practice footer. The report of 13 February 2025 carries “disclosed for Qsuper and not for medico-legal use” on every page. **If that appears on this report it is compromised on its face.** The covering email raises it; confirm it has been understood.

4. SEQUENCE: serve the AO3 role description on the Regulator BEFORE the report is commissioned. Her amended List of Documents has 52 items and no role description of any kind. If the report reasons from a document she has never seen, it surfaces through an expert instead of through disclosure. **Serve the document, not its provenance** — no mention of the request for medical information, the exclusion, or 31 July 2026.

5. The commencement date. Question 1.3(g) says “from 2019”. **Have a document for it** — first payslip, letter of appointment, or service record — before it is relied on in the appeal.

B · WHEN THE REPORT ARRIVES

6. NO DRAFT WAS REQUESTED — struck 17 August. Offering to review a draft still looks like a chance to shape the report, so the report arrives issued. **If a fact in it is wrong — a date, a name, which document said what, an attachment reference — correct it by writing to Dr Krishnaiah after it issues, facts only, and keep the letter.** Do not comment on any opinion, do not suggest a conclusion, do not ask for anything to be strengthened.

7. THE PREPARATION AS A SYMPTOM — the first thing to look for. His report of 13 February 2025 lists, **under symptoms:** “obsessive rumination, fear driven thoughts, **extensive researching, and planning on fact findings for work-related issues including reading up on policies, procedures, and legislations**”, and records that the proceedings “have consumed his whole life severely affecting his functioning.” ⇒ **A seven-page instruction, a schedule and seven attachments, prepared by the patient, is on its face the behaviour he already coded as pathology.** Watch for any sentence treating the preparation of the case as a symptom, or the litigation as the barrier to return. **It is an incapacity marker, it is usable by the employer, and s 32(5)(c) excludes injury from action taken in connection with the compensation application.** The covering email now states he is self-represented and prepared the instruction himself.

8. “AGGRAVATING” MUST NOT BE USED OF THE APPEAL OR THE EMPLOYMENT PROCESS.

Aggravation is statutory: s 32 makes it a compensable injury in its own right, and **s 32(5)(c)** then **excludes it**. In the capacity process the same word reads as **deterioration**. The correct register is “**perpetuating**” or “**maintaining**” — persistence, not worsening, and it does not compete with causation of onset.

9. PROTECT THE SLEEP STRAND. Sleep runs the length of the case and it runs from **shift work: 16 Nov 2023** “*Poor sleep. Shift work. Cannot work/ do shifts if he does not get a good sleep*” → the 7-hour break → **3 Jul 2026** “*maintaining a regular sleep routine remains central to symptom management.*” **Any sentence attributing current sleep disturbance to the appeal cuts that thread at its strongest point.**

10. THE FIVE POSITIVE CHECKS. Does it **list every document reviewed** (1.5 — the Review Decision safeguard) · **state which Part B sources** the causation opinion rests on (Part D(d)) · **answer under the letter's numbering** (Part D(a)) · avoid “*unfair dismissal*” and “*bullying*” — his own 2025 register · and avoid the practice footer.

11. WHAT IS RAISABLE AND WHAT IS NOT. Terminology and factual error are raisable — “*aggravating*”, a wrong date, a misattributed document. **Anything that is opinion is not**, unless it misstates the record.

C · STANDING DISCIPLINES

12. Say nothing about which exhibit holds what, or about the span of the production. The letter and the schedule state only what the Regulator asserts — a history from 26 October 2022 — and nothing about what the file does or does not contain. **Do not add it back in correspondence.** It is their assertion to prove.

13. Assume everything sent to the practice is produced to the Regulator under the notice it holds. That is why the instruction and the covering email carry no strategy and no characterisations.

14. The employer-facing material goes in the covering note, not to the clinician: QH-POL-210's whole-organisation test and onus · that the Director who asks about “*restrictions or modifications*” approved three · that Item 3(a) explains why the Health Service is “*not aware of any concerns*” · the September 2025 conduct/July 2026 medical asymmetry.

15. WHAT THE REPORT MUST NOT DO. Say he cannot fulfil the inherent requirements · answer the accommodation question in the negative · opine on working with any named person or reporting line · express any view on whether management action was reasonable · attribute the injury to a course of management conduct generally rather than to the rostering and fatigue strand specifically.

D · STILL OUTSTANDING

16. The three movement forms are IN — Attachment 5, stitched with an index page. Nothing further needed.

17. The April 2025 individual monthly statistics — 269 to 444 calls per shift at about 29% of full time. They answer “*he kept working for fourteen months*”, which the capability checklist does not reach.

18. Who is performing the duties, which shifts, from when — in writing. Currently colleague report only.